

Data Management Plan (CDCL.10 / CDCM.03)

Frasier Digital, LLC — RFQ 75D301-26-Q-00146, Modernized VAERS Reporting Application

1. Public health data to be collected or generated during performance

The application collects **VAERS adverse-event report submissions** from public and healthcare-provider submitters using the data elements of the current VAERS form (as furnished by CDC at kickoff), plus: uploaded medical-record documents (Phase 1 scope: medical records and vaccine documents related to the administration or adverse event), free-text supplemental information, customer-satisfaction survey responses (site navigation + post-submission), and operational usage metrics supporting the PWS performance targets (completion rates, abandonment, submission timing). The data may include PII and PHI. Frasier Digital generates no independent research data; all data is collected on CDC's behalf, inside CDC's environment, as an instrument of the VAERS program.

2. Standards

Data capture conforms to the **Government-furnished VAERS data element definitions, business rules, and integration requirements** (PWS Section 9, provided at kickoff); outputs are **VAERS-compatible structured data** with documented mappings (PWS Task 1.9) for transmission to existing VAERS systems as directed by CDC. Metadata and export formats use open, machine-readable standards (JSON/CSV/XML per the furnished integration specifications). Records management follows NARA-approved CDC schedules (CDCH.14).

3. Access and sharing — mechanisms, limitations, protections

All collected data is **CDC-owned and resides exclusively within the CDC-managed FedRAMP-authorized Azure environment**; Frasier Digital asserts no rights and imposes no limitations (FAR 52.227-17 unlimited Government rights; HHS/CDC retain unrestricted rights to all data). Access is need-to-know, role-based, and logged, under the controls of the SSP and the ATD/ATO. **This DMP proposes no public access mechanism:** VAERS submission data contains PII/PHI protected by the Privacy Act and HIPAA; public availability of VAERS data remains governed by CDC's existing de-identified VAERS data release processes, which are outside this contract's scope — this justification is provided per the CDCL.10 instruction that protections precluding accessibility be explained. De-identified, aggregate operational metrics (e.g., satisfaction and performance reporting) are delivered to CDC and may be released at CDC's discretion. Custom-developed source code (non-data) is delivered to the CDC-recognized repository with Government rights per FAR 52.227-17 (Amendment 0001, Q&A 91).

4. Archiving and long-term preservation

Data persists in CDC-managed storage under CDC's NARA-approved records schedules; the application implements retention/disposition configuration as directed by CDC records authorities rather than contractor-defined schedules. At transition-out or contract end, all data, documentation, and configurations are delivered per PWS Task 4 with certification of sanitization of any contractor-side copies per NIST SP 800-88 (no contractor-side copies are

anticipated: development uses synthetic data and Government data never leaves the CDC environment). Long-term preservation responsibility rests with CDC as data owner; we deliver the technical documentation (schemas, mappings, metadata) sufficient for preservation and future use.

5. Costs

No separate data-management costs: DMP implementation is inherent in the proposed firm-fixed price (schema/mapping documentation, retention configuration, transition deliverables). No archiving infrastructure is purchased — storage is the CDC-furnished environment.

Lifecycle maintenance: the DMP will be updated throughout performance as data collections evolve (e.g., Phase 2 upload expansion if exercised), per CDCL.10.